



KNOW YOUR CYCLE

A practical, evidence-based guide to your hormones, your symptoms
and your fertility signs — and to knowing when to ask for help.

ELSA · OSTEOPATH & WOMEN'S HEALTH PRACTITIONER

BEFORE YOU BEGIN

CLARITY, NOT CONFUSION.

Your cycle tells a story every month. This guide teaches you to read it: what is typical, what is worth tracking, and which signs deserve a doctor's attention. It does not ask you to be perfect. It asks you to be curious.

THREE WAYS TO USE THIS GUIDE

01

Read

Chapters 1 to 6 explain how the cycle works, in plain language, with the numbers clinicians use.

02

Track

Chapter 7 and the printable worksheets help you collect one to three cycles of useful data.

03

Prepare

Chapter 8 turns your notes into a one-page summary you can bring to a consultation.

MEDICAL SAFETY NOTE

This guide is educational. It helps you observe patterns, ask better questions and prepare for care. It does not diagnose, treat or replace your doctor, and it is not a method of contraception. Elsa is an osteopath and women's health practitioner, not a medical doctor. The clinical figures quoted here come from the guidelines listed on page 27, checked in October 2026.

GET URGENT CARE NOW IF YOU HAVE

- Bleeding that soaks a pad or tampon every hour for two hours or more, or heavy bleeding with dizziness, fainting or a racing heart.
- Sudden, severe pelvic or abdominal pain, especially if you could be pregnant, or pain with fever.
- Fever, vomiting, a sunburn-like rash or feeling faint while using a tampon or cup: remove it and seek care.
- Thoughts of harming yourself. Call your local emergency number or a crisis line (988 in the US, 116 123 in the UK and Ireland, 3114 in France).

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ABOUT THE AUTHOR

I'm Elsa, an osteopath (M.Ost., University College of Osteopathy, London) and women's health practitioner with more than seven years of clinical practice. I wrote this guide because I spent years with painful periods, PMS and no clear explanation. My work sits alongside your doctor's, never instead of it.

THE GUIDE IN TEN POINTS

01

Your cycle is a **vital sign**: changes in timing, flow or symptoms carry information about your overall health.

02

In adults, a cycle of **24 to 38 days** with bleeding of up to **8 days** is within typical limits. Teenagers can range from 21 to 45 days.

03

Ovulation rarely falls exactly on day 14. The **luteal phase is steadier**; the phase before ovulation varies most.

04

The **fertile window lasts about six days**: the five days before ovulation and the day of ovulation.

05

Cervical mucus can announce ovulation; basal temperature only **confirms it afterwards**; apps alone often get the day wrong.

06

Period pain that stops you living your life is **common, not normal**, and deserves assessment.

07

Heavy bleeding has concrete signs: changing protection every one to two hours, clots the size of a coin, bleeding more than seven days.

08

Severe premenstrual mood symptoms may be **PMDD**, a recognised and treatable condition, diagnosed by tracking two cycles.

09

Sleep, enough food, stress and movement support ovulation. **Under-eating** and chronic stress can switch it off.

10

One to three months of simple notes make any consultation **faster, clearer and more useful**.

01

THE CYCLE AS A VITAL SIGN

YOUR CYCLE IS NOT BACKGROUND NOISE.

A period is the visible part of a much larger rhythm involving the brain, the ovaries, the uterus, the thyroid, the stress system, sleep and nutrition. When something shifts in that system, the cycle often shows it first.

EXPERT LENS

The American College of Obstetricians and Gynecologists (ACOG) recommends treating the menstrual cycle as a vital sign, alongside pulse or blood pressure, because patterns in timing, flow and symptoms can reveal problems early.¹ The International Federation of Gynecology and Obstetrics (FIGO) defines the limits clinicians use to describe a typical cycle.^{2,3}

WHAT YOUR RHYTHM CAN REVEAL

Rhythm

Cycle length, regularity, missed periods and sudden changes.

Flow

Number of bleeding days, heaviness, clots, spotting between periods.

Signals

Pain, mood, sleep, skin, digestion, energy, libido, and when in the cycle they appear.

A BETTER QUESTION THAN “IS THIS NORMAL?”

Ask instead: **Is this typical for me, and has it changed?** One imperfect cycle after travel, illness or a stressful month is common. A new pattern that repeats for two or three cycles is information worth acting on.

You do not need to track perfectly. You need enough data to recognise your own rhythm, so that a change stands out.

WHAT IS A TYPICAL CYCLE?

Ranges are orientation, not a verdict. They describe most people, and they are what a doctor will compare your notes against.

PARAMETER	ADULTS (FIGO) ^{2,3}	FIRST YEARS AFTER THE FIRST PERIOD (ACOG) ¹
Cycle length	24 to 38 days	21 to 45 days
Bleeding duration	Up to 8 days	Up to 7 days
Regularity	Shortest and longest cycle differ by 7 days or less (9 days or less under 26 and over 41)	Irregular cycles are common; a single cycle over 90 days still needs checking
Volume	Judged by its impact on your life, not by a number	Changing protection more often than every 1 to 2 hours is heavy

HOW TO COUNT

Day 1

The first day of **real bleeding**, the kind that needs protection. Light spotting before it does not count.

Cycle length

From day 1 of one period to the day before the next period starts.

Ovulation

Usually 10 to 16 days **before** the next period. In a large study of 600,000 cycles, only 13% lasted exactly 28 days. ⁴

Regularity

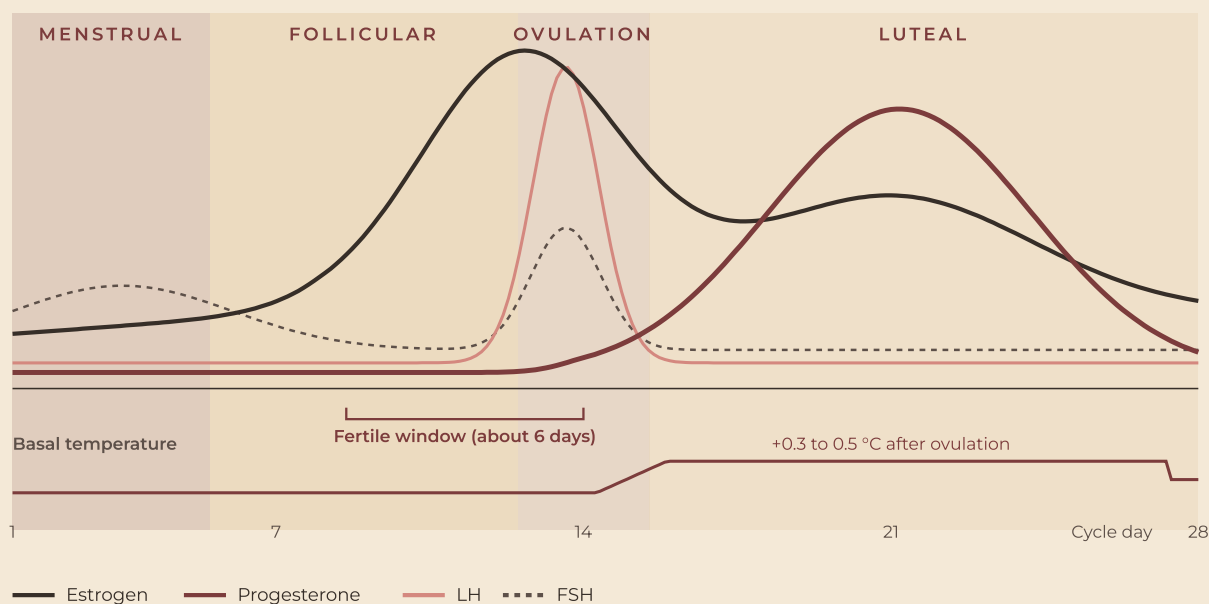
Compare your shortest and longest cycle over several months. One cycle tells you little; six tell you a lot.

YOUR CYCLE IS A MONTHLY DATA SET. YOU DO NOT NEED TO BE PERFECT AT TRACKING — JUST CONSISTENT ENOUGH TO SEE YOUR OWN PATTERN.

02 HORMONES AND PHASES

TWO CYCLES, ONE RHYTHM.

There are really two cycles running together. In the **ovary**, a follicle matures, releases an egg, then becomes the corpus luteum. In the **uterus**, the lining sheds, rebuilds and prepares. The four “seasons” used in this guide are a teaching map laid over both.



Schematic 28-day cycle, not to scale. Real cycles vary: the follicular phase lengthens or shortens; the luteal phase is steadier.

Before ovulation

Rising estrogen rebuilds the uterine lining and makes cervical mucus wetter. This phase can last from about 10 to 30 days, which is why cycle length varies.⁴

After ovulation

The corpus luteum produces progesterone, which stabilises the lining and lifts basal temperature. This phase usually lasts 10 to 16 days, then hormones fall and the period starts.

THE FOUR SEASONS, MADE SIMPLE.

01

Menstrual · days 1 to 5 or so

The lining sheds. Hormones are at their lowest. Energy may dip. Cramping is common; pain that disrupts your life is not something to accept in silence.

02

Follicular · until ovulation

Estrogen rises. Many people notice more energy, sociability or focus. Mucus gradually becomes creamier, then wetter.

03

Ovulatory · a few days

The LH surge triggers ovulation. Mucus is often clear, slippery and stretchy. Some feel a one-sided twinge (mittelschmerz) or notice higher libido.

04

Luteal · about 10 to 16 days

Progesterone rises, then falls if no pregnancy occurs. Temperature stays higher. Premenstrual symptoms, if any, appear in the last days.

IMPORTANT NUANCE

Premenstrual symptoms are not a sign that your hormones are “out of balance”.

Research suggests they come from an increased sensitivity of the brain to normal hormone changes. Stress, poor sleep and alcohol can amplify them, which is why the foundations in chapter 6 help.

WHAT THE EVIDENCE SAYS ABOUT “CYCLE SYNCING”

Phases are physiology, not personality types. A 2020 meta-analysis found exercise performance may be only trivially lower in the early follicular phase, with low-quality evidence; researchers advise a personal approach rather than rigid rules.²¹ Use phases to understand your patterns, never to limit yourself.

WHO DOES WHAT.

A simple orientation, not a lab interpretation. Hormone tests are only meaningful when timed to your cycle and read by a clinician.

FSH · follicle-stimulating hormone

Released by the pituitary gland early in the cycle, it recruits follicles in the ovary.

Estrogen (estradiol)

Made by the growing follicle. Builds the uterine lining and produces fertile cervical mucus. Supports bone, skin and mood.

LH · luteinising hormone

Surges sharply about one to one and a half days before ovulation and triggers the release of the egg. Ovulation tests detect it.⁸

Progesterone

Made after ovulation by the corpus luteum. Stabilises the lining, raises basal temperature and has a calming, sometimes sedating effect.

Prostaglandins

Local messengers that make the uterus contract. High levels drive cramping, and sometimes loose stools during the period. Anti-inflammatory painkillers (NSAIDs) work by lowering them.

Thyroid, prolactin, cortisol, insulin

Not “cycle hormones”, but each can disturb ovulation. This is why a doctor investigating irregular cycles often checks them.

GOOD TO KNOW

Bleeding is not proof of ovulation. In an **anovulatory cycle** there is no egg and no progesterone rise, yet bleeding can still happen, often irregular or heavier. Temperature charts and mucus observations help tell the difference.

03

FERTILITY SIGNS

FERTILITY IS VISIBLE, IF YOU KNOW WHERE TO LOOK.

Fertility awareness means observing biological signs, not trusting a date predicted by an app. The fertile window is not the whole month: it lasts about **six days**, the five days before ovulation and the day of ovulation itself.^{5,6}

01

Cervical mucus

Becomes wet, slippery and stretchy as ovulation approaches. It helps sperm survive for up to about five days.

ANNOUNCES

02

Basal temperature

Rises by about 0.3 to 0.5 °C after ovulation, under the effect of progesterone, and stays up until the period.⁷

CONFIRMS AFTERWARDS

03

LH tests

Detect the LH surge in urine. After a positive test, ovulation usually follows within about two days.^{6,8}

PREDICTS, BRIEFLY

WHY COMBINING SIGNS WORKS BETTER

Each sign answers a different question. Mucus says *ovulation may be coming*. An LH test says *it is probably within two days*. Temperature says *it has happened*. Together they let you reconstruct each cycle with confidence.

TRYING TO CONCEIVE?

Intercourse every one to two days during the days of slippery mucus gives the best chances; the highest probability is around the last day of slippery mucus (the “peak day”).⁶ There is no need to time things to the hour. If you are under 35 and have tried for 12 months, or 35 or over and have tried for 6 months, ask for a fertility assessment, sooner if cycles are irregular or absent.⁶

A PRACTICAL CERVICAL MUCUS MAP.

Notice the **sensation** at the vulva during the day (dry, damp, wet, slippery), then look at what is on the paper when you wipe before using the toilet. Sensation matters as much as appearance.³⁰

WHAT YOU NOTICE	HOW IT MAY LOOK OR FEEL	WHAT IT MAY MEAN
Dry or nothing	No sensation, nothing visible	Lower fertility, usually just after the period
Sticky or tacky	Thick, pasty, crumbly, breaks easily	Fertility low but starting to rise
Creamy	Smooth, lotion-like, white or cloudy	Fertility rising; estrogen increasing
Wet, slippery, stretchy	Clear, like raw egg white, stretches between fingers, lubricative feeling	Highest fertility; ovulation is near
Sudden return to sticky or dry	The slippery sensation stops	The last slippery day is the “peak day”; ovulation usually occurs within a day or two of it

WHAT CAN CHANGE MUCUS

Hormonal contraception, vaginal infections, lubricants, semen, breastfeeding, perimenopause, some medicines (such as antihistamines) and dehydration.

SEE A PROFESSIONAL IF DISCHARGE

has a strong or fishy smell, is green, grey or frothy, looks like cottage cheese, or comes with itching, burning, pain, bleeding after sex or fever.

TEMPERATURE, TESTS AND APPS.

Basal body temperature

- Take it on waking, before getting up, at about the same time, with a thermometer accurate to 0.05 °C.
- Look for a sustained shift of about 0.3 to 0.5 °C over the previous six readings.
- Alcohol, illness, short sleep and travel can blur the chart.

Urine LH tests

- Start testing a few days before your expected ovulation, at the time of day the instructions recommend.
- False positives happen in about 7% of cycles.⁶
- With PCOS (persistently high LH) or in perimenopause, results can be misleading.

ABOUT TRACKING APPS

Apps that predict ovulation from calendar dates alone identified the right day in at most about one cycle in five.⁶ Use an app as a notebook for your own observations, not as an oracle. Check its privacy policy: cycle data is sensitive health data.

CONTRACEPTION NOTE

Do not use fertility signs to avoid pregnancy unless you have been taught a specific method by a qualified instructor and follow its rules exactly. With typical use, ACOG reports 12 to 24 pregnancies per 100 women in the first year; with perfect use, fewer than 1 to 5. An app on its own is not a method.

ON HORMONAL CONTRACEPTION

With the combined pill, patch or ring, ovulation is suppressed and the monthly bleed is a **withdrawal bleed**, not a true period, so fertility signs cannot be read.²³ The hormonal IUD and some progestogen-only pills often do not stop ovulation, so cycles and signs may continue.

04

SYMPTOMS AS SIGNALS

TRACK SYMPTOMS AS INFORMATION, NOT IDENTITY.

Symptoms are not personal failures. They are patterns. The goal is to connect their timing with your cycle phase and with sleep, food, stress, movement and life events.

Pain

Cramps, pelvic or back pain, pain with sex, with bowel movements or passing urine, ovulation pain.

Bleeding

Heavy days, clots, flooding, spotting between periods, bleeding after sex.

Mood

Anxiety, irritability, sadness, anger, low motivation, feeling overwhelmed.

Energy and sleep

Fatigue, insomnia, waking at night, afternoon crashes, low resilience.

Skin and hair

Jawline acne, oily skin, hair thinning or extra facial and body hair.

Digestion

Bloating, constipation, diarrhoea, nausea, cravings.

THREE QUESTIONS FOR EACH SYMPTOM

1. **When?** Before bleeding, during it, around ovulation, after ovulation, or at random times?
2. **How much?** Score it from 0 to 10, and note what it stopped you doing.
3. **What helped?** Rest, heat, painkillers, food, movement, nothing?

SCORING PAIN CONSISTENTLY

0-3 noticeable, life goes on · **4-6** distracting, you push through or need painkillers · **7-10** you cancel plans, miss school or work, cannot sleep, or painkillers do not help. Scores of 7 or more, repeated, deserve a medical opinion.

PMS, PMDD, OR SOMETHING ELSE?

Many people notice mild changes in the days before a period. When symptoms become severe, they deserve a name and proper care.

	PREMENSTRUAL SYNDROME (PMS)	PREMENSTRUAL DYSPHORIC DISORDER (PMDD)
What it is	Physical and emotional symptoms in the luteal phase that are bothersome	A severe, mainly mood-based condition recognised in the DSM-5-TR ¹⁹
How common	Very common in a mild form	About 2 to 6% of people who menstruate ¹⁹
Typical signs	Bloating, breast tenderness, irritability, fatigue, cravings	Marked depression, anxiety, anger or mood swings that disrupt relationships, work or study
Timing	Symptoms appear in the final week before bleeding, improve within a few days of the period starting, and are minimal or absent in the week after.	
Diagnosis	Based on daily symptom ratings over at least two cycles . This is exactly what the worksheets in chapter 7 help you collect. Effective treatments exist, from lifestyle measures to medication. ²⁰	

RULE OUT LOOK-ALIKES

If symptoms are present all month and only worsen before the period, this may be **premenstrual exacerbation** of another condition (depression, anxiety, migraine, irritable bowel). The timing is what tells them apart.

PLEASE DO NOT WAIT

PMDD is linked to a higher risk of suicidal thoughts. If you have thoughts of harming yourself, at any point in your cycle, contact your local emergency number or a crisis line today (988 in the US, 116 123 in the UK and Ireland, 3114 in France).

05

WHEN TO ASK FOR MEDICAL HELP

PAIN IS COMMON. BEING DISMISSED SHOULD NOT BE.

Use these lists to decide how quickly to act. If a period is late or missing and you could be pregnant, **take a pregnancy test first.**¹²

TODAY · EMERGENCY CARE

- Soaking a pad or tampon every hour for two hours or more, or heavy bleeding with dizziness, fainting, breathlessness or a racing heart.
- Sudden severe pelvic pain, one-sided pain with bleeding, shoulder-tip pain or fainting, especially with a possible pregnancy (risk of ectopic pregnancy).
- Pelvic pain with fever, or symptoms of toxic shock syndrome with a tampon or cup in place.

IN THE COMING WEEKS · BOOK AN APPOINTMENT

- Period pain that makes you miss school, work or sleep, or that painkillers no longer control.^{13,15}
- Pain that is getting worse over time, pain with sex, or cyclical pain when passing stools or urine.
- Heavy bleeding (see next page), or tiredness and breathlessness that may signal iron deficiency.
- Bleeding between periods or after sex.¹⁰
- Cycles regularly shorter than 24 days or longer than 38 days, or cycle lengths varying by more than 7 to 9 days.^{2,3}
- No period for 3 months if your cycles were regular, or 6 months if they were irregular.¹²
- New acne, excess hair growth or hair thinning together with irregular cycles; milky discharge from the nipples.
- Any bleeding one year or more after your final period (postmenopausal bleeding).²⁹

“THIS SYMPTOM IS AFFECTING MY DAILY LIFE. I WOULD LIKE TO UNDERSTAND THE POSSIBLE CAUSES AND WHAT ASSESSMENT IS APPROPRIATE.”

HEAVY BLEEDING, ABSENT PERIODS, SPECIAL AGES.

Is my bleeding heavy?

Doctors define heavy menstrual bleeding as blood loss that interferes with your physical, social, emotional or material quality of life.¹⁰ Practical signs:⁹

- Soaking through one or more pads or tampons every hour for several hours.
- Needing two products at once, or changing at night.
- Clots the size of a large coin (2.5 cm) or bigger.
- Bleeding that lasts more than 7 days.

Why it matters

Heavy periods are a leading cause of **iron deficiency**, with or without anaemia: fatigue, breathlessness, hair loss, poor concentration. Ask whether a blood count and ferritin are appropriate.^{10,11}

About 1 in 5 teenagers assessed for heavy periods have an underlying **bleeding disorder**, most often von Willebrand disease. Mention easy bruising, nosebleeds or family history.¹¹

Teenagers

Cycles can be irregular for a few years after the first period. Still seek advice if there is no period by age 15 (or 3 years after breast development), if any cycle lasts more than 90 days, or if bleeding lasts more than 7 days.¹

Perimenopause

From the 40s, cycles become more variable. A persistent difference of 7 days or more between cycles, then gaps of 60 days or more, are typical stages.²⁴ Very heavy, very frequent or between-period bleeding still deserves a check.

MISSING PERIODS ARE A SYMPTOM, NOT A DIAGNOSIS

After pregnancy, the most common causes are low energy availability (under-eating, intense training, weight loss), stress, PCOS, thyroid problems, high prolactin and, more rarely, early ovarian insufficiency. Absent periods can affect long-term bone health, so they deserve an assessment rather than waiting.^{12,22}

CONDITIONS WORTH KNOWING BY NAME.

A language page, not a diagnostic one: it helps you describe your experience and understand what a doctor may look for.

Endometriosis

Tissue similar to the uterine lining grows outside the uterus. Affects about 10% of women and girls of reproductive age.¹⁶ Painful periods, chronic pelvic pain, pain with sex, bowel or bladder symptoms, fatigue, sometimes infertility. Pain severity does not match the extent of disease. Diagnosis often takes several years; current guidelines allow a clinical diagnosis and imaging, and a normal scan does not rule it out.^{13,14,15}

PCOS

Affects about 10 to 13% of women.¹⁷ In adults, diagnosis needs two of: irregular or absent ovulation, signs of excess androgens (acne, hair growth, blood tests), polycystic ovaries on ultrasound or a high AMH. In teenagers, ultrasound and AMH are not used. It has metabolic, mood and long-term health dimensions. A 2026 international consensus proposed a new name, polyendocrine metabolic ovarian syndrome (PMOS); the 2023 criteria still apply.¹⁸

Adenomyosis

Lining-like tissue within the muscle of the uterus. Heavy, painful periods and a feeling of pelvic pressure, more often diagnosed after 35. Detected by ultrasound or MRI.

Fibroids and polyps

Usually benign growths of the uterus. Can cause heavy or prolonged periods, bleeding between periods, pressure or frequent urination.

Thyroid and prolactin

An under- or overactive thyroid, or high prolactin, can disturb cycle length, flow, energy, mood, temperature and fertility. Simple blood tests check them.

Functional hypothalamic amenorrhea

The brain pauses ovulation when energy intake does not meet the demands of training, weight loss or stress. A diagnosis made after other causes are excluded; recovery relies on enough food and rest, with medical follow-up.²²

HORMONAL HEALTH IS NOT A HACK.

The cycle responds to the whole body. The aim is not to control every variable, but to create conditions that make ovulation, recovery and calm more likely. No fear, no perfection: what can you *add* to support your body?

Enough energy

Eating enough, regularly, is the foundation. Chronic under-fuelling is one of the most common reasons ovulation stops.²²

STRONG EVIDENCE**Sleep**

Regular sleep and morning light support the body clock, appetite regulation and pain tolerance.

GOOD EVIDENCE**Stress physiology**

The stress system and the reproductive system talk to each other. Chronic strain can delay ovulation and amplify premenstrual symptoms.

GOOD EVIDENCE**Movement**

Regular activity, including strength training and walking, supports metabolism, mood and period pain. Adapt intensity to how you feel, not to a calendar.

GOOD EVIDENCE**Heat and pain relief**

Local heat and, if suitable for you, anti-inflammatory painkillers taken at the first sign of pain are first-line for period cramps. Ask a pharmacist or doctor.

GOOD EVIDENCE**Alcohol and tobacco**

Both worsen sleep and premenstrual symptoms, and smoking is linked to more painful periods and earlier menopause.

GOOD EVIDENCE**A NOTE ON SUPPLEMENTS**

Few supplements have solid evidence for cycle symptoms, and some interact with medicines or the pill. Discuss them with a doctor or pharmacist rather than stacking products.

A CYCLE-SUPPORTIVE PLATE, WITHOUT DIET CULTURE.

- **Protein** at each meal, for satiety, tissue repair and steadier energy.
- **Fibre and colour** (vegetables, fruit, legumes, whole grains) for digestion and a diverse gut microbiota.
- **Carbohydrates as fuel**, especially around training and in the luteal phase, when appetite naturally rises.
- **Healthy fats** (oily fish, olive oil, nuts, seeds) to absorb vitamins A, D, E and K.
- **Iron-rich foods** if your periods are heavy, paired with vitamin C to improve absorption.

IF YOU ARE TRYING TO CONCEIVE

No single diet has been shown to improve natural fertility.⁶ One measure is clearly proven: **folic acid, 400 µg a day**, started at least one month before conception and continued in early pregnancy, to reduce the risk of neural tube defects.²⁶ Your doctor may advise a higher dose in some situations.

LIFE STAGES THAT CHANGE THE PICTURE

Coming off hormonal contraception

Most people ovulate again within a few months. Symptoms that the pill was masking (acne, pain, irregular cycles) may return. If periods have not returned after three months, seek advice.

After pregnancy

Breastfeeding often delays the return of ovulation and periods, but ovulation can occur before the first period. Mucus and temperature are harder to interpret.

The first years after the first period

Irregular cycles are common as the brain-ovary dialogue matures. Severe pain or very heavy periods are not part of “growing up”.

Perimenopause

Often from the early to mid 40s: shorter or longer cycles, heavier periods, sleep and mood changes, hot flushes. Support and treatments exist.

07

YOUR FIRST MONTHS OF TRACKING

FIVE THINGS, ONE LINE A DAY.

Keep it light. Track one to three cycles. You are looking for patterns, not perfection.

01

Bleeding

0 none · S spotting · 1 light · 2 medium · 3 heavy · 4 very heavy. Add “C” for clots.

02

Mucus

D dry · St sticky · Cr creamy · W wet · Sl slippery or stretchy. Circle the peak day.

03

Temperature (optional)

On waking, before getting up, same time each day. Note anything that may disturb it.

04

Symptoms

Pain, mood, energy, sleep, digestion, skin: a 0 to 10 score for the one or two that matter most to you.

05

Context

Stress, travel, illness, alcohol, intense training, new medication, big life events. Context explains many “strange” cycles.

TINY TRACKING RULE

If tracking makes you anxious, simplify: bleeding and your one main symptom are enough. If you have ever struggled with your relationship with food or your body, skip any tracking of weight or food quantities.

ONE CYCLE, ONE PAGE

Cycle start date: _____ Cycle no.: ____

DAY	DATE	BLEEDING	MUCUS	TEMP °C	PAIN 0-10	MOOD 0-10	ENERGY 0-10	NOTES AND CONTEXT
1								
2								
3								
4								
5								
6								
7								
8								
9								
10								
11								
12								
13								
14								
15								
16								
17								
18								
19								
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21								
22								
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24								
25								
26								
27								
28								
29								
30								
31								
32								
33								
34								
35								
36								
37								
38								

Bleeding: 0 · S · 1 · 2 · 3 · 4 (+C clots) | Mucus: D · St · Cr · W · SI (circle the peak day) | Print one page per cycle, or copy the columns into your notes app.

SEE YOUR PATTERN EMERGE.

AT THE END OF EACH CYCLE	CYCLE 1	CYCLE 2	CYCLE 3
Start date			
Cycle length (days)			
Bleeding days / heaviest day			
Products per day on heaviest day			
Clots? Night changes?			
Spotting between periods?			
Mucus peak day			
Temperature rise day			
Luteal phase (rise to next period)			
Highest pain score / days missed			
Main symptom and its timing			
Best energy days			
Lowest energy days			
Sleep and stress overall			

WHAT CHANGED THIS MONTH, AND WHAT DO I WANT TO ASK?

08

PREPARING FOR A CONSULTATION

BRING THE PATTERN, NOT JUST THE PAIN.

Appointments are short. An organised story helps your clinician understand you faster and order the right tests. You do not need perfect answers, just a clear starting point.

Timeline

When did this start? Any trigger: a contraception change, pregnancy or loss, infection, new medication, a move, a stressful period?

Cycle pattern

Average and range of cycle length, bleeding days, heaviest day, spotting, missed periods. Bring worksheets 1 and 2.

Top three symptoms

What affects your life most, its score, and when in the cycle it happens. What it stops you doing.

Medical context

Diagnoses, medicines and supplements, contraception history, surgeries, previous tests, family history (endometriosis, PCOS, bleeding disorders, thyroid).

Your goal

Less pain, lighter periods, understanding irregular cycles, coming off the pill, trying to conceive, managing perimenopause, or simply understanding your body. Saying it out loud helps the clinician focus.

QUESTIONS YOU CAN ASK

- ☐ What could explain these symptoms, and what would you check first?
- ☐ Is a blood test (blood count, ferritin, thyroid, hormones timed to my cycle) or an ultrasound useful for me?
- ☐ What are my treatment options, and what are their side effects?
- ☐ What should make me come back sooner?

FOR MY APPOINTMENT ON ____ / ____ /

MY MAIN CONCERN, IN ONE SENTENCE

MY GOAL FOR THIS APPOINTMENT

AVERAGE CYCLE LENGTH / RANGE

BLEEDING DAYS / HEAVIEST DAY

SYMPTOM 1 · SCORE · TIMING IN CYCLE

SYMPTOM 2 · SCORE · TIMING IN CYCLE

SYMPTOM 3 · SCORE · TIMING IN CYCLE

DAYS MISSED (WORK, SCHOOL, SOCIAL) PER CYCLE

CURRENT MEDICINES, SUPPLEMENTS, CONTRACEPTION

WHAT I HAVE ALREADY TRIED, AND THE RESULT

RELEVANT FAMILY HISTORY

COULD I BE PREGNANT? LAST PERIOD STARTED

MY QUESTIONS

THE INNER RHYTHM METHOD.

This guide is the starting point. In one-to-one online sessions, in English or French, we go further at your pace, in four steps.

01

Understand

We go back over your history, your cycle and your symptoms to see what is really going on.

02

Build the basics

We adjust a few habits: food, sleep, digestion, stress. Not many, but ones you can keep.

03

Listen to your body

You learn to read your cycle and notice what you need at each point in it.

04

Find your balance

You keep what works and carry on by yourself, without depending on me.

WHAT THIS IS

- Body literacy and practical, personalised support.
- Help preparing for, and making sense of, medical appointments.
- A space where your symptoms are taken seriously.

WHAT THIS IS NOT

- A diagnosis, a prescription or a substitute for your doctor.
- A magic supplement protocol or a weight-loss programme.
- A place where pain is minimised.

READY TO UNDERSTAND WHAT YOUR BODY HAS BEEN TRYING TO SAY?

Book a free, no-commitment 15–20 minute call.

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A FEW WORDS MADE SIMPLER.

Amenorrhea

Absence of periods: 3 months without bleeding after regular cycles, 6 months after irregular ones.

Anovulatory cycle

A cycle without ovulation. Bleeding can still occur.

AUB

Abnormal uterine bleeding: any bleeding outside typical limits of timing, regularity, duration or volume.

Basal body temperature

Resting temperature on waking. Rises after ovulation.

Corpus luteum

The structure left in the ovary after ovulation. It makes progesterone.

Dysmenorrhea

Painful periods. Primary: without an underlying condition. Secondary: caused by one, such as endometriosis.

Fertile window

About six days per cycle when intercourse can lead to pregnancy.

Heavy menstrual bleeding (HMB)

Blood loss that affects quality of life. Replaces the older term “menorrhagia”.

LH surge

The rapid rise of luteinising hormone that triggers ovulation.

Luteal phase

From ovulation to the next period, when progesterone is higher.

Ovulation

Release of an egg from the ovary. The egg survives about 12 to 24 hours.

Peak day

The last day of slippery, most fertile-type cervical mucus.

PMDD

Premenstrual dysphoric disorder: severe premenstrual mood symptoms that disrupt life.

Withdrawal bleed

Bleeding caused by stopping hormones, for example during the pill-free week. Not a true period.

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CLOSING NOTE

YOUR BODY IS NOT THE PROBLEM. IT IS A SYSTEM TO UNDERSTAND.

Start with observation. Add context. Ask better questions. Get support when your symptoms affect your life. This is the beginning of body literacy.



Educational content, checked against clinical guidelines current in October 2026. Not a diagnostic tool, a contraceptive method or a substitute for medical care. Seek urgent help for severe pain, fainting, heavy bleeding with dizziness, fever, a possible pregnancy complication, or any symptom that feels unsafe.

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